جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : خديجه صلاح الدين السيد ابراهيم
المحافظة : القاهره **الحي / المدينة :** الحسين **العنوان :** ~~
رقم البطاقة : 28208151604264 **رقم التليفون :** 01276008577 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 15-08-1982 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: خديجه صلاح الدين السيد ابراهيم
National ID: 28208151604264

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



خديجه
صلاح الدين السيد ابراهيم
٥١ ش النصر - ش الاهرام - بيجام
شبرا الخيمة أول - القليوبية

٢٨٢ ٠٨ ١٥ ١٦ ٠٤٢ ٦٤

KJ3777803

٢٨٢٠٨١٥١٦٠٤٢٦٤ ٢٠٢٤/٠١
حاصلة على دبلوم صنايع

انثى مسلمة متزوجة
وليد عبدالمطلب عبدالمطلب على غانم
١١ البطاقة سارية حتى ٢٠٣٠/١٢/٣١

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MDT form

A- Patient ID:
1- Name : [redacted] Sex: Female
2-ID number: 28208151604264
3- Medical number: 83081352
Patient Age : 40 Y

C.C.A.P.
C.C.C

Family history	-VE	Phone	01206951482
BMI		NRS Score	
GROUP	A	marital status	
Menopausal status	PRE	Co morbidity	
Date of MDT	12/20/2022	REASON OF MDT	POST LT MRM

Clinical data

Breast Imaging :

Mets Workup :
CTCAP6/2022-->FREE

BX:	IHC:	ER:	PR:	Her 2 Neu :	KI67 :
LT IDC G11		8/8	8/8	1VE	20%

T	N	M
4D	2	0

Sonomammography
6/2022
LT MULTI FOCAL MALIGNANT MASS BIRADS 5
PATHOLOGICAL LT AXILLARY LNS

Recommendations:
LT BREAST CANCER POST NACT,MRM SPECIMEN:
TOTAL TUMOR REGRESSION IN A PREVIOUSLY DIAGNOSED INVASIVE CARCINOMA
FREE NIPPLE AND AREOLA WITH DERMAL SUPPURATIVE INFLAMMATORY REACTION
FREE DEEP MARGIN (LN 1/10)
Path. Par. Resp. YPT0 YPN1

MDT OUTCOME & PLAN

For: .PORT to Lt. chest wall, paraclav. & axilla.
.TDM1 for a total of 1 yr anti Her2 (14 cycles)
.Endocrinal TTT (Zoladex/AI for 5yrs).

C.C. 12/12/22

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	:	Request No.	: 13005241
Patient No.	: 28208151604264	Request Date	: 10/12/2022
Poli No.	: 3710/22	Verification Date	: 19/12/2022
Sex	: Female	Financial Category	:
Age	: 40 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data :
Left breast cancer post NACT
Nature of the specimen : Right MRM specimen collected at 10/12/22
Gross Examination:
Left MRM specimen measured 19x17x7 cm , with covering skin ellpse measured 21x14 cm
- Tumor extent : An ill defined rubbery greyish white tumor bed site
- Size: 2x1.5x1cm
- Skin involvement: Nipple & areola were unremarkable grossly with reddish skin lesion oozing yellowish fluid
- Distance from margins: 0.2 cm away from deep margin
- Lymph nodes number and size: 10 LNs up to 1 cm .

Microscopic examination:
- Histological type: Total tumor regression in a previously diagnosed carcinoma
- Lymph node involvement: Positive (1/10)
- Skin involvement : Free nipple and areola
-Skin lesion showed dermal suppurative inflammatory reaction
- Resection Margins: Free deep margin
- Histologic response to therapy: Pathologic partial response

Diagnosis
Left Breast cancer , post NACT , MRM specimen :
-Total tumor regression in a previously diagnosed Invasive carcinoma
-Free nipple and areola , with dermal suppurative inflammatory reaction
-Free deep margin
-Lymph nodes.Psitive (1/10)
-Pathologic partial response

Verified by :
dr.esraa magdy

Printing Date : 21/12/2022 12:51:35

Printing By : marwa mohamed youss

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذة / نادية صلاح الدين السيد ابراهيم	Request No.	: 1300851
Patient No.	: 28208151604264	Request Date	: 06/06/20
Path No.	: 1552/22	Verification Date	: 16/06/20
Sex	: Female	Financial Category	: [illegible]
Age	: 39 Y	Department	: Out Patie
Referreing Doc.	:	IHC No.	:
		Registered by	: Ali mosta

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast mass lesion.

Nature of the specimen:
Tru cut biopsy collected 6-6-2022 (13:00)

Gross Examination:
- Four greyish white ,fatty and hemorrhagic tissue cores up to 1.5 cm .Totally submitted .
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA syst
PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ:No
- Biomarkers:
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)
Ki 67: 20 % nuclear reaction in tumor cells
- Positive control showed appropriate reactivity

Diagnosis:
Left breast mass lesion , Tru cut biopsy:
- Invasive duct carcinoma grade II
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).

قسم الباثولوجي

Verified by :
walaa ghoniem

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذة / خدوجة صلاح الدين السيد ابراهيم	Request No.	13008516
Patient No.	28208151601264	Request Date	06/06/202
Path No.	1552/22	Verification Date	16/06/202
Sex	Female	Financial Category	لا - [illegible]
Age	39 Y	Department	Out Patior
Referreing Doc.		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

- HER-2: Positive (score 3+)
- Ki67: Positive in 20% of tumor cells.

Prof.Dr.Gamal Abdela
Dr. Israa Elgendy

Dr. Walaa Ghonaim

Verified by :
walaa ghonlem

Immunohistochemistry
[[DOCTYPE:IHC]]

Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/06/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: :

A known case of breast cancer.

1. *Chest:*

- Left breast Heterogeneously enhanced parenchyma with overlying thickened skin. Bilateral axillary enlarged lymph nodes the largest at left side measures about 13 X 9 mm.
- No sizable pulmonary nodules.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. *Abdomen and pelvis:*

- The liver is enlarged showing homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- *Left breast Heterogeneously enhanced parenchyma with overlying thickened skin and bilateral axillary enlarged lymph nodes.*
- *No CT evidence of pulmonary or hepatic metastatic deposits.*

Dr. Ayat Nadi

Dr. M. Gomaa, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

NASSER INSTITUTE
ONCOLOGY CENTER

NUCLEAR MEDICINE UNIT

Patient Name: Khadiga Solaheldeen Elsayed. Date: 3/3/2022

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

- Warm uptake is seen involving the right femur and right tibia likely post internal fixation and intervention for clinical and radiological correlation as well as follow up.
- Otherwise, no definite hot spot that account for osseous deposits could be detected allover the rest of surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- *No definite evidence of metastatic osseous deposits.*
- *Clinical and radiological correlation of the right femur and tibia is recommended.*

Thanks for reference
Nuclear Medicine Unit

DR/Huda Fathy, MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]





ERM036722449

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 29/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- Right UIQ area of asymmetrical density with no underlying spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast UIQ area of asymmetry appreciated more in CC view for follow up after 6 months by single view mammogram (BIRADS 3)

Yours truly

Signature Dr. Asmaa Zenhom

29/08/2024 3:53 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/06/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

1. ***Chest:***

- Left breast Heterogeneously enhanced parenchyma with overlying thickened skin. Bilateral axillary enlarged lymph nodes the largest at left side measures about 13 X 9 mm.
- No sizable pulmonary nodules.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
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2. ***Abdomen and pelvis:***

- The liver is enlarged showing homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- ***Left breast Heterogeneously enhanced parenchyma with overlying thickened skin and bilateral axillary enlarged lymph nodes.***
- ***No CT evidence of pulmonary or hepatic metastatic deposits.***

Dr. Ayat Nadi

Dr. M. Gomaa, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 43 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

FOLLOW UP RIGHT BREAST MAMMOGRAPHY WITH BILATERAL ULTRASOUND

INDICATION:

- Left MRM

TECHNIQUE:

- Right breast MLO and CC views
- Bilateral ultrasound

REPORT:

- Fairly dense parenchyma, (ACR B), with the focal tissue island still seen in the upper midline aspect of the breast.
- No spiculated masses or grouped microcalcifications.
- Complementary U/S also showed no solid or cystic masses, only underlying condensation of parenchyma.
- Normal skin thickness and nipple-areola complex.
- Right nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions, and free infra and supra clavicular regions.

OPINION:

- Left MRM with free operative bed.
- Stationary right benign finding, (BIRADS 2), for follow up.

Best regards,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية التخصصية
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/11/2022
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Indication:

left breast pathologically proved carcinoma on chemotherapy.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast no spiculated mass lesion seen in UOQ, stationary course of UOQ suspicious microcalcifications.
- Normal skin thickness, and nipple-areola complex of the right breast.
- One left still noted with cortical thickening reaching to 5mm in level I.
- supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast pathologically proved carcinoma on chemotherapy with good response in ultrasound but still noted UOQ malignant microcalcifications and still noted one pathological axillary lymph node at level I, (BIRADS 6)
- Normal right breast (BIRADS 1)
-

Signature Dr. Nehal Hamdy

23/11/2022 12:35 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Presidential Program for Women's health

Name: Khadija salaheddin elsayed

Date: 7/7/2022

Targeted Bilateral axillary Ultrasound

Clinical summary: left breast pathologically proved multicentric IDC

Technique

High resolution ultrasound with Color Doppler.

Detailed Findings

Left breast

- Dense fibro glandular breast parenchyma.
- Second look ultrasound showed multicentric malignant looking variable size mass lesions scattered all over the area between 10 o'clock till 5 o'clock, involving the axillary tail of the breast. Scattered dilated ducts of involved areas are noted suggesting intraductal extension.
- Thickened skin/nipple/areolar complex. Reaching 7 mm
- Multiple Enlarged Pathological lymph nodes with thickened cortex reaching 10 mm with eccentric hilum , involving Level I and II including the infraclavicular lymph nodes, with 2 suspicious visualized supraclavicular lymph nodes, showing cortical thickness of 2 mm.

Right breast

- Dense fibroglandular breast parenchyma.
- No solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Axillary non-specific nodes.

Conclusion:

Left breast (BIRADS 6) multicenteric IDC, with pathological axillary lymph nodes involving Level I and II with suspicious supraclavicular lymph nodes .
Right breast (BIRADS 1) non-specific axillar lymph nodes with Max cortical thickness of 2.8 mm .

DR, Eman Elmaraghi, FEBR

Ultrasound
[[DOCTYPE:ULTRASOUND]]

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وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/06/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 969	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast multiple irregular spiculated hypoechoic mass lesions seen at 2 , 4 , 11 and 12 o'clock largest seen at 2 o'clock measure 3.7 x 2 cm and 12 o'clock measures 3 x 1.2 cm
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary level I, II and III lymph nodes largest at level I measures 2.8 x 1.4 cm
- No supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast multifocal malignant looking mass lesions (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Normal right breast (BIRADS 1)

Yours truly

Signature Dr. Aya Deabes

05/06/2022 03:13 م

Mammogram
[[DOCTYPE:MAMMOGRAM]]

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

NASSER INSTITUTE
ONCOLOGY CENTER
خطابات أورام

NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Khadiga Salaheldeen Elsayed. *Date: 3/8/2022*

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Warm uptake is seen involving the right femur and right tibia likely post internal fixation and intervention for clinical and radiological correlation as well as follow up.
- Otherwise, no definite hot spot that account for osseous deposits could be detected allover the rest of surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- ***No definite evidence of metastatic osseous deposits.***
- ***Clinical and radiological correlation of the right femur and tibia is recommended.***

Thanks for reference
Nuclear Medicine Unit

DR/Huda Fathy, MD

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel : 22039164 - 22039356 Fax : 22039 236

Bone Scan
[[DOCTYPE:BONE_SCAN]]

وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام

صحة و السكان
راكز الطبية المتخصصة
دار السلام

٧/١٢/٧

اسم المريض رباعي : خديجة صالح المصباحي

رقم الملف الطبي : ٢٥٠

نوع الوجبة :

تاريخ اعداد الوجبة :

وقت اعداد الوجبة :

eport

EF	67	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.6	2-3.7cm
LVESD	2.8	2-4cm	LA	3.2	2-4cm
FS	37	25-45%	RV	—	0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO CUSP	—	1.5-2.6cm

LV dimensions نارمل

LV wall thickness نارمل

LV diastolic function نارمل

SWMA نارمل

LA and Ao root dimensions نارمل

Mitral Valve نارمل

Aortic valve نارمل

Tricuspid valve نارمل ع trivial TR, normal RVP.

Pulmonary valve نارمل

Right side and systolic function نارمل

Intracardiac masses and effusion نارمل

Mohammed Anwar, Msc cardiology.

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 29/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- Right UIQ area of asymmetrical density with no underlying spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast UIQ area of asymmetry appreciated more in CC view for follow up after 6 months by single view mammogram (BIRADS 3)

Yours truly

Signature Dr. Asmaa Zenhom

29/08/2024 3:53 PM

ERM036722449





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
3- Medical number: 83081352
1- Name : خديجة صلاح الدين السيد ابراهيم Sex: Male
2-Id number: 28209151604264 Patient Age: 40 Y

Family history -VE
Phone 01204881482
BMI
BRS Score
GROUP A
marital status
Menopausal status PRE
Co-morbidity
Date of MDT 12/29/2022
REASON OF MDT POST LT MRM

Clinical data

Breast Imaging :

Mets Workup :
CTCAP6/2022—>FREE

BX:	IHC:	ER:	PR:	Her 2 Neu :	KI67 :
LT IDC Gr2		6/8	8/8	+VE	20%

T	N	M
4D	2	0

Sonomammography
6/2022
LT MULTI FOCAL MALIGNANT MASS BIRADS 5
PATHOLOGICAL LT AXILLARY LNS

Recommendations:
LT BREAST CANCER POST NACT,MRM SPECIMEN:
TOTAL TUMOR REGRESSION IN A PREVIOUSLY DIAGNOSED INVASIVE CARCINOMA
FREE NIPPLE AND AREOLA WITH DERMAL SUPPURATIVE INFLAMMATORY REACTION
FREE DEEP MARGIN (LN 1/10)
Path. Par. Resp. YPT0 YPN1

MDT OUTCOME & PLAN

For: .PORT to Lt. chest wall, paraclav. & axilla.
.TDM1 for a total of 1 yr anti Her2 (14 cycles)
.Endocrinal TTT (Zoladex/AI for 5yrs).

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

MDT FORM 23/11/2022 GA

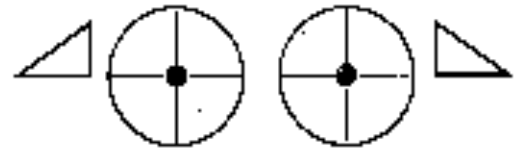
Name		MRN	
		ID	
Age		Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
8|8	7|8	+ve	20%

R L



Sonomammography:

Mets Workup:

Metastatic screen
recmd CT C & AP

TNM

was clinically node
neg with right breast involvement.

Recommendations:

for adjuvant ... as per initial MDT
Rt MRM
Rt SLNB → requested

MDT OUTCOME & PLAN

For left MRM per initial decision.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
لمنطقة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name. Khadiga Salah Al Din Al Sayed Ibrahim	Date : 29/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- Right UIQ area of asymmetrical density with no underlying spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast UIQ area of asymmetry appreciated more in CC view for follow up after 6 months by single view mammogram (BIRADS 3)

Yours truly

Signature Dr Asmaa Zenhom

29/08/2024 3:53 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الأشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/06/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

1. ***Chest:***

- Left breast Heterogeneously enhanced parenchyma with overlying thickened skin. Bilateral axillary enlarged lymph nodes the largest at left side measures about 13 X 9 mm.
- No sizable pulmonary nodules.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is enlarged showing homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- ***Left breast Heterogeneously enhanced parenchyma with overlying thickened skin and bilateral axillary enlarged lymph nodes.***
- ***No CT evidence of pulmonary or hepatic metastatic deposits.***

Dr. Ayat Nadi

Dr. M. Gomaa, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 43 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

FOLLOW UP RIGHT BREAST MAMMOGRAPHY WITH BILATERAL ULTRASOUND

INDICATION:

- Left MRM

TECHNIQUE:

- Right breasl MLO and CC views
- Bilateral ultrasound

REPORT:

- Fairly dense parenchyma, (ACR B), with the focal tissue island still seen in the upper midline aspect of the breast.
- No spiculated masses or grouped microcalcifications.
- Complementary U/S also showed no solid or cystic masses, only underlying condensation of parenchyma.
- Normal skin thickness and nipple-areola complex.
- Right nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions, and free infra and supra clavicular regions.

OPINION:

- Left MRM with free operative bed.
- Stationary right benign finding, (BIRADS 2), for follow up.

Best regards,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE:MAMMOGRAM]]

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Radiology Department

وزارة الصحة و السكان
لمدنة المراكز الطبية المتخصصة
مستشفي أورام دار السلام
ادارة الاشعة

Patient Name: Khadiga Salah A Din Al Sayed Ibrahim	Date : 23/11/2022
Exam : MAMMOGRAPHY BOTH	Age 40 Year(s)
Medical ID: 2288/2022	Pat. ID. 12669

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Indication:

left breast pathologically proved carcinoma on chemotherapy.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast no spiculated mass lesion seen in UOQ, stationary course of UOQ suspicious microcalcifications
- Normal skin thickness, and nipple-areola complex of the right breast.
- One left still noted with cortical thickening reaching to 5mm in level I
- supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN

Opinion:

- Left breast pathologically proved carcinoma on chemotherapy with good response on ultrasound but still noted UOQ malignant microcalcifications and still noted one pathological axillary lymph node at level I, (BIRADS 5)
- Normal right breast (BIRADS 1)
-

Signature Dr. Nehal Hamdy

23/11/2022 12:35 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Presidential Program for Women's health

Name: Khadija salaheddin elsayed

Date: 7/7/2022

Targeted Bilateral axillary Ultrasound

Clinical summary. left breast pathologically proved multicentric IDC

Technique

High resolution ultrasound with Color Doppler.

Detailed Findings

Left breast

- Dense fibro glandular breast parenchyma.
- Second look ultrasound showed multicentric malignant looking variable size mass lesions scattered all over the area between 10 o'clock till 5 o'clock, involving the axillary tail of the breast. Scattered dilated ducts of involved areas are noted suggesting intraductal extension.
- Thickened skin/nipple/areolar complex. Reaching 7 mm
- Multiple Enlarged Pathological lymph nodes with thickened cortex reaching 10 mm with eccentric hilum , involving Level I and II including the infraclavicular lymph nodes, with 2 suspicious visualized supraclavicular lymph nodes, showing cortical thickness of 2 mm

Right breast

- Dense fibroglandular breast parenchyma.
- No solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Axillary non-specific nodes.

Conclusion:

Left breast (BIRADS 6) multicenteric IDC, with pathological axillary lymph nodes involving Level I and II with suspicious supraclavicular lymph nodes . Right breast (BIRADS 1) non-specific axillar lymph nodes with Max cortical thickness of 2.8 mm .

DR, Eman Elmaraghi, FEBR

Scanned with CamScanner
MULTITYPE SONOGRAPHY

Ministry Of Health
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Radiology Departments

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/06/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 989	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast multiple irregular spiculated hypoechoic mass lesions seen at 2 , 4 , 11 and 12 o'clock largest seen at 2 o'clock measure 3.7 x 2 cm and 12 o'clock measures 3 x 1.2 cm
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary level I, II and III lymph nodes largest at level I measures 2.8 x 1.4 cm
- No supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast multifocal malignant looking mass lesions (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Normal right breast (BIRADS 1)

Yours truly

Signature Dr. Aya Deabes

05/06/2022 03:13 م

Mammogram
[[DOCTYPE:MAMMOGRAM]]

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

NASSER INSTITUTE
ONCOLOGY CENTER
خطابات أورام

NUCLEAR MEDICINE UNIT
وحدة الطب النووى

Patient Name: Khadiga Salaheldeen Elsayed. *Date: 3/8/2022*

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

- Warm uptake is seen involving the right femur and right tibia likely post internal fixation and intervention for clinical and radiological correlation as well as follow up.
- Otherwise, no definite hot spot that account for osseous deposits could be detected allover the rest of surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- ***No definite evidence of metastatic osseous deposits.***
- ***Clinical and radiological correlation of the right femur and tibia is recommended.***

Thanks for reference
Nuclear Medicine Unit

DR/Huda Fathy, MD

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351 Kurniesh El Nile Aghakhan - Shoubra Cairo Tel.: 22039164 - 22039356 Fax : 22039 236

Bone Scan
[[DOCTYPE:BONE_SCAN]]

وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مركز اورام دار السلام

٧/١٧

اسم المريض رباعي :
رقم الملف الطبي :
نوع الوحدة :
تاريخ اعداد الوجبة :
وقت اعداد الوجبة :

(١٥)

Report

EF	67	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO R	2.6	2-3.7cm
LVESD	2.8	2-4cm	LA	3.2	2-4cm
FS	37	25-45%	RV	—	0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP	—	1.5-2.6cm

LV dimensions نارمل

LV wall thickness نارمل

LV diastolic function نارمل

SWMA نارمل

LA and Ao root dimensions نارمل

Mitral Valve نارمل

Aortic valve نارمل

Tricuspid valve نارمل مع trivial TR, نارمل PVR

Pulmonary valve نارمل

Right side and systolic function نارمل

Intracardiac masses and effusion نارمل

Mohammed Anwar, Msc cardiology.

Echo
[[DOCTYPE:ECHO]]